TABLE 2–1. Basic principles of GPM

Principle	Comment
1. Offer psychoeducation.	Don't hesitate to inform patients about what you know—or don't know—about their diagnosis and its treatment. Also, feel free to give advice when you feel it will be instructive.
2. Be active, not reactive.	Being responsive assures patients that you are interested and involved; do not catastrophize.
3. Be thoughtful.	You are a container for your patient's anxiety and a role model for "thinking first."
4. The relationship is real as well as professional.	Both aspects are necessary.
5. Convey that change is expected.	Its absence raises questions about the treatment's value.
6. Foster accountability.	Encourage being responsible for behavior, most notably for within-treatment failures to remember or implement the lessons learned in prior sessions.
7. Maintain a focus on life outside of treatment.	Stay informed about outside relationships, emphasize the value of structured vocational activities.
8. Be flexible, pragmatic, and eclectic.	Your responses and interventions are determined by the patient and good sense.

4. The Therapy Relationship Is Real (Dyadic) As Well As Professional

Acknowledge your mistakes (e.g., "I misunderstood," "I should have…," "If I could do it over, I'd…"). Fear of making mistakes can rob both you and the patient of an essential humanizing process (see also the principle "Change Is Expected" below). Accept idealization, but do not encourage unrealistic expectations ("I'm flattered that you think…," "My experience was different than you think…"). Self-disclosure can be helpful or harmful: do not deny the obvious or that the patient affects you, and always consider whether self-disclosure will help the patient (see Table 2–2).

5. Change Is Expected

Inform patients with BPD that the usual course of their disorder is one of gradual improvement. Underscore that whether improvement will occur depends

TABLE 2–2. Self-disclosure by clinicians in treating borderline personality disorder

Anonymity is a myth—what varies is the accuracy of a patient's perceptions; we affect this.

Ways that self-disclosure can help:

- Normalizes feelings and beliefs that patients feel shamed by
- Encourages hope
- Establishes authenticity
- Gives permission to feel, say, act

on their taking on an active role. They can count on you to assist. Identify as long-term goals that they may be able to achieve satisfactory work and partnerships but that those also will depend on their efforts and chance.

6. Accountability

Hold patients responsible for what they say while retaining the leavening attitudes that your patient's mistakes, intolerance, hostility, or offensive behaviors are understandable and can change. Within sessions, remind patients of prior discussions; ask whether they had forgotten the conclusions reached, and, if they have, question how they could do that. Do not reflexively try to diminish guilt or shame; sometimes experiencing and acknowledging such feelings should be applauded. When these feelings can be borne without action or dissociation, they are preconditions and incentives for remorse, apologies, reconciliation, and change. The converse side of this is the clinician's need to be accountable for his or her own mistakes, feelings, or attitudes. Be ready to own these.

7. Focus on Life Outside of Treatment

Know what is happening in your patients' primary relationships and, given their instability, discuss the interpersonal perceptions and reactions as they recur. Most of their emotional (e.g., anger) and behavioral (e.g., self-harm) problems are related to interpersonal events. They need to recognize this. You can help them manage situational stressors. Sometimes wise advice is sufficient. Sometimes you should invite input from others (e.g., talks with members of the family). Interpersonal hypersensitivity can be buffered by having structured vocational activities. Clinicians should emphasize the message of "work before love" and encourage and aid vocational initiatives. Even by 6 months, borderline patients should be expected to have some school, work, or domestic responsibilities (see Table 2–3). Although a trust-